Fibrinogen, concentrate from human plasma: Drug information - UpToDate

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Contributor Disclosures

For additional information see ”Fibrinogen, concentrate from human plasma: Patient drug information” and ”Fibrinogen, concentrate from human plasma: Pediatric drug information”

For abbreviations, symbols, and age group definitions show table

Brand Names: US

- Fesilty;
- Fibryga;
- RiaSTAP

Brand Names: Canada

- Fibryga;
- RiaSTAP

Pharmacologic Category

- Blood Product Derivative

Dosing: Adult

Acquired fibrinogen deficiency

Acquired fibrinogen deficiency (Fibryga): IV: 4 g as a single dose; may repeat dose as needed in bleeding patients when plasma fibrinogen level is ≤200 mg/dL or thromboelastometry FIBTEM A10 is ≤10 mm (or equivalent values generated by other viscoelastic testing methods). Dose may be individualized based on plasma fibrinogen levels or viscoelastic testing, severity of bleeding, body weight, or patient’s clinical condition.

Congenital fibrinogen deficiency

Congenital fibrinogen deficiency: IV: Note: Maintain a target fibrinogen level of 100 mg/dL for minor bleeding and 150 mg/dL for major bleeding.

When baseline fibrinogen level is known:

Fesilty, Fibryga: Dose (mg/kg) = [Target level (mg/dL) - measured level (mg/dL)] **divided by** 1.8 (mg/dL per mg/kg body weight)

RiaSTAP: Dose (mg/kg) = [Target level (mg/dL) - measured level (mg/dL)] **divided by** 1.7 (mg/dL per mg/kg body weight)

When baseline fibrinogen level is not known: 70 mg/kg

Dosing: Kidney Impairment: Adult

There are no dosage adjustments provided in the manufacturer's labeling. However, need for dosage adjustment unlikely as high molecular weight precludes filtration at the glomerulus (Ref).

Dosing: Liver Impairment: Adult

There are no dosage adjustments provided in the manufacturer's labeling.

Dosing: Older Adult

Refer to adult dosing; has not been studied.

Dosing: Pediatric

(For additional information see "Fibrinogen, concentrate from human plasma: Pediatric drug information")

Acquired fibrinogen deficiency; treatment of acute bleeding

Acquired fibrinogen deficiency; treatment of acute bleeding: Note: Dosing must be individualized for each patient based on the extent of bleeding, laboratory values, clinical condition of the patient, fibrinogen concentration, or viscoelastic properties of fibrin-based clot.

Children <12 years: Fibryga: IV: 70 mg/kg; may repeat dose as needed in bleeding patients when plasma fibrinogen ≤200 mg/dL or thromboelastometry FIBTEM A10 ≤10 mm (or equivalent values measured by other viscoelastic testing methodology).

Children ≥12 years and Adolescents: Fibryga: IV: 50 mg/kg; may repeat dose as needed in bleeding patients when plasma fibrinogen ≤200 mg/dL or thromboelastometry FIBTEM A10 ≤10 mm (or equivalent values measured by other viscoelastic testing methodology).

Congenital fibrinogen deficiency; treatment of acute bleeding

Congenital fibrinogen deficiency; treatment of acute bleeding: Note: Dosing must be individualized for each patient based on the extent of bleeding, laboratory values, clinical condition of the patient, and target fibrinogen concentration. The recommended target fibrinogen concentration is 100 mg/dL for minor bleeding and 150 mg/dL for major bleeding.

Initial dose:

When baseline fibrinogen concentration is known:

RiaSTAP: Infants, Children, and Adolescents (Ref): IV:

Dose (mg/kg) = [Target fibrinogen concentration (mg/dL) – measured fibrinogen concentration (mg/dL)] **divided by 1.7** (mg/dL per mg/kg body weight)

Fibryga:

Children <12 years: IV:

Dose (mg/kg) = [Target fibrinogen concentration (mg/dL) – measured fibrinogen concentration (mg/dL)] **divided by 1.4** (mg/dL per mg/kg body weight)

Children ≥12 years and Adolescents: IV:

Dose (mg/kg) = [Target fibrinogen concentration (mg/dL) – measured fibrinogen concentration (mg/dL)] **divided by 1.8** (mg/dL per mg/kg body weight)

When baseline fibrinogen level is not known: Fibryga, RiaSTAP: Infants, Children, and Adolescents: IV: 70 mg/kg.

Repeat doses: Adjust dose based on laboratory values and condition of patient. Administer additional doses if the fibrinogen concentration falls below 80 mg/dL for minor bleeding and below 130 mg/dL for major bleeding until hemostasis is achieved.

Dosing: Kidney Impairment: Pediatric

There are no dosage adjustments provided in manufacturer's labeling.

Dosing: Liver Impairment: Pediatric

There are no dosage adjustments provided in manufacturer's labeling.

Adverse Reactions

The following adverse drug reactions and incidences are derived from product labeling unless otherwise specified.

>10%:

Cardiovascular: Atrial fibrillation (29%)

Hematologic & oncologic: Anemia (16%)

Nervous system: Delirium (15%)

1% to 10%:

Cardiovascular: Thromboembolic complications (9%; including acute myocardial infarction, arterial thrombosis, cerebrovascular accident, deep vein thrombosis, disseminated intravascular coagulation, ischemic hepatitis, mesenteric ischemia, peripheral ischemia, portal vein thrombosis, pulmonary embolism, thrombophlebitis, thrombosis, transient ischemic attacks)

Gastrointestinal: Nausea (>5%), vomiting (>5%)

Hematologic & oncologic: Hematologic abnormality (fibrin D-dimer increased), thrombocytopenia (4%), thrombocytosis (>5%)

Hepatic: Abnormal liver function (7%)

Hypersensitivity: Facial swelling

Nervous system: Headache (>1%)

Neuromuscular & skeletal: Back pain, limb pain

Renal: Acute kidney injury (8%), kidney failure (5%)

Respiratory: Pneumonia (5%)

Miscellaneous: Fever (>1%)

Frequency not defined:

Cardiovascular: Ischemia (digital foot)

Dermatologic: Erythema of skin, pruritus

Postmarketing:

Hypersensitivity: Hypersensitivity reaction (including anaphylaxis)

Nervous system: Chills

Contraindications

Hypersensitivity, including anaphylaxis, to fibrinogen concentrate, human plasma-derived products, or any component of the formulation.

Warnings/Precautions

Concerns related to adverse effects:

• Hypersensitivity: Hypersensitivity reactions (eg, hives, generalized urticaria, chest tightness, wheezing, hypotension, anaphylaxis) may occur. In the event of hypersensitivity reactions, treatment should be discontinued immediately.

• Thrombotic events: Thrombosis may occur spontaneously in patients with acquired or congenital fibrinogen deficiency with or without fibrinogen replacement therapy. Thromboembolic events have been reported in patients receiving fibrinogen concentrate. Consider potential risk of thrombosis with use.

Dosage form specific issues:

• Human plasma: Product of human plasma; may potentially contain infectious agents which could transmit disease (eg, viruses and theoretically the Creutzfeldt-Jakob disease [CJD]). Screening of donors, as well as testing and/or inactivation or removal of certain viruses, reduces the risk. Infections thought to be transmitted by this product should be reported to the manufacturer.

• Polysorbate 80: Some dosage forms may contain polysorbate 80 (also known as Tweens). Hypersensitivity reactions, usually a delayed reaction, have been reported following exposure to pharmaceutical products containing polysorbate 80 in certain individuals (Isaksson 2002; Lucente 2000; Shelley 1995). Thrombocytopenia, ascites, pulmonary deterioration, and renal and hepatic failure have been reported in premature neonates after receiving parenteral products containing polysorbate 80 (Alade 1986; CDC 1984). See manufacturer's labeling.

Other warnings/precautions:

• Appropriate use: Not indicated for the treatment of dysfibrinogenemia.

Dosage Forms Considerations

Strengths expressed with approximate values. Consult individual vial labels for exact potency within each vial.

Dosage Forms: US

Excipient information presented when available (limited, particularly for generics); consult specific product labeling.

Injection, powder for reconstitution:

Fesilty: Fibrinogen Concentrate (Human)-chmt ~1 g [exact potency labeled on vial]

Fibryga: ~1 g; ~2 g [exact potency labeled on vial]

RiaSTAP: 900-1300 mg [contains albumin (human); exact potency labeled on vial]

Generic Equivalent Available: US

No

Pricing: US

Solution (reconstituted) (Fesilty Intravenous)

1 g (0 Price provided is per milligram): $1.32

Disclaimer: A representative AWP (Average Wholesale Price) price or price range is provided as reference price only. A range is provided when more than one manufacturer's AWP price is available and uses the low and high price reported by the manufacturers to determine the range. The pricing data should be used for benchmarking purposes only, and as such should not be used alone to set or adjudicate any prices for reimbursement or purchasing functions or considered to be an exact price for a single product and/or manufacturer. Medi-Span expressly disclaims all warranties of any kind or nature, whether express or implied, and assumes no liability with respect to accuracy of price or price range data published in its solutions. In no event shall Medi-Span be liable for special, indirect, incidental, or consequential damages arising from use of price or price range data. Pricing data is updated monthly.

Dosage Forms: Canada

Excipient information presented when available (limited, particularly for generics); consult specific product labeling.

Injection, powder for reconstitution:

Fibryga: ~1 g [exact potency labeled on vial]

RiaSTAP: 900-1300 mg [contains albumin (human); exact potency labeled on vial]

Administration: Adult

IV: For IV administration only; infuse at a rate not exceeding 5 mL/minute (congenital fibrinogen deficiency) or 20 mL/minute (acquired fibrinogen deficiency). Solution should be infused at room temperature. Flush line with NS before and after administration; do not allow any blood to enter syringe due to risk of fibrin clot formation. Do not administer with other products or IV solutions. Administration should be completed within 8 hours (RiaSTAP) or 4 hours (Fesilty, Fibryga) after reconstitution. For RiaSTAP, filter reconstituted solution with a 17-micron filter (not supplied) into an appropriate syringe before administration. For Fesilty and Fibryga, no filter is required if prepared using the Nextaro transfer device; if prepared using an Octajet transfer device (how supplied prior to May 2025), then filter reconstituted solution with the supplied 17-micron particle filter into an appropriate syringe before administration.

Preparation for Administration: Adult

Fesilty: Prior to reconstitution, warm both powder and diluent vials (SWFI) to room temperature. Open the Nextaro transfer device (provided) by removing the lid but keep in the blister package; do not touch the spike. Place diluent vial on a surface and snap the blue part of the Nextaro transfer device onto the diluent vial, then remove the blister package. Do not twist the transfer device. Place powder vial on a surface, turn diluent vial with attached transfer device upside down, and snap into place on powder vial, allowing diluent to automatically transfer. Ensure a vacuum is maintained by attaching Nextaro transfer device to the diluent vial first, then to the powder vial. Following diluent transfer, keep diluent vial attached and gently swirl until powder is completely dissolved (~3 minutes); discard if not fully dissolved within 30 minutes. Do not shake. Solution should be clear to slightly opalescent; discard if cloudy or contains particulates. Remove transfer device from diluent vial in 2 parts by turning counterclockwise. Attach a syringe to the Luer lock connector on the Nextaro transfer device and draw solution into the syringe. Remove syringe from transfer device and discard powder vial and device. If more than 1 vial is needed for the dose, reconstitute each vial using a new Nextaro transfer device. Refer to manufacturer's labeling for additional preparation instructions.

Fibryga:

If prepared using the Octajet transfer device (how supplied prior to May 2025): Prior to reconstitution, warm both powder and SWFI to room temperature; if water bath is used for warming, the water bath should not exceed 37°C (98°F). Reconstitute with 50 mL of SWFI; gently swirl until dissolved (~5 to 10 minutes), do not shake. Filter reconstituted solution with the supplied 17-micron particle filter into an appropriate syringe before administration. Solution should be almost colorless and slightly opalescent; discard if cloudy or contains particulates. Refer to manufacturer's labeling for additional preparation instructions.

If prepared using the Nextaro transfer device: Prior to reconstitution, warm both powder and diluent vials (SWFI) to room temperature; if using a water bath, the temperature should not exceed 37°C (98°F). Open the Nextaro transfer device (provided) by removing the lid but keep in the blister package; do not touch the spike. Place diluent vial on a surface and snap the blue part of the Nextaro transfer device onto the diluent vial, then remove the blister package. Place powder vial on a surface, turn diluent vial with attached transfer device upside down and snap into place on powder vial, allowing diluent to automatically transfer. Ensure a vacuum is maintained by attaching Nextaro transfer device to the diluent vial first, then to the powder vial. Following diluent transfer, keep diluent vial attached and gently swirl until powder is completely dissolved (~5 to 10 minutes); do not shake. Solution should be almost colorless and slightly opalescent; discard if cloudy or contains particulates. Remove transfer device from diluent vial in 2 parts by turning counterclockwise. Attach a syringe to the Luer lock connector on the Nextaro transfer device and draw solution into the syringe. Remove syringe from transfer device and discard powder vial and device. Refer to manufacturer's labeling for additional preparation instructions.

RiaSTAP: Transfer 50 mL of SWFI into fibrinogen concentrate vial. Gently swirl until dissolved; do not shake. Filter reconstituted solution with a 17-micron filter (not supplied) into an appropriate syringe before administration. Solution should be colorless and clear to slightly opalescent; discard if cloudy or contains particulates.

Administration: Pediatric

IV: For IV administration only; rate dependent upon indication. Solution should be infused at room temperature. Flush line with NS before and after administration; do not allow any blood to enter syringe due to risk of fibrin clot formation. Do not administer with other products or IV solutions.

Acquired fibrinogen deficiency: Fibryga: Infuse at a rate ≤20 mL/minute.

Congenital fibrinogen deficiency: Fibryga, RiaSTAP: Infuse at a rate ≤5 mL/minute.

Preparation for Administration: Pediatric

IV:

Fibryga:

Preparation using the Octajet transfer device (how supplied prior to May 2025): Prior to reconstitution, warm both powder and SWFI (not provided) to room temperature; if water bath is used for warming, the water bath should not exceed 37°C (98°F) and water should be prevented from coming into contact with the rubber stoppers or caps of the bottles. Reconstitute with 50 mL SWFI, gently swirl until dissolved (~5 to 10 minutes); do not shake. Filter reconstituted solution with the supplied 17-micron particle filter into an appropriate syringe before administration. Solution should be almost colorless and slightly opalescent; discard if cloudy or contains particulates. See product labeling for additional information.

Preparation using the Nextaro transfer device: Prior to reconstitution, warm both powder and diluent vials (SWFI) to room temperature; if using a water bath, the temperature should not exceed 37°C (98°F). Open the Nextaro transfer device (provided) by removing the lid but keep in the blister package; do not touch the spike. Place diluent vial on a surface and snap the blue part of the Nextaro transfer device onto the diluent vial, then remove the blister package. Place powder vial on a surface, turn diluent vial with attached transfer device upside down, and snap into place on powder vial, allowing diluent to automatically transfer. Ensure a vacuum is maintained by attaching Nextaro transfer device to the diluent vial first, then to the powder vial. Following diluent transfer, keep diluent vial attached and gently swirl until powder is completely dissolved (~5 to 10 minutes); do not shake. Solution should be almost colorless and slightly opalescent; discard if cloudy or contains particulates. Remove transfer device from diluent vial in 2 parts by turning counterclockwise. Attach a syringe to the Luer lock connector on the Nextaro transfer device and draw solution into the syringe. Remove syringe from transfer device and discard powder vial and device. Refer to manufacturer's labeling for additional preparation instructions.

RiaSTAP: Transfer 50 mL of SWFI into fibrinogen concentrate vial. Gently swirl until dissolved; do not shake. Filter reconstituted solution with a 17-micron filter (not supplied) into an appropriately sized syringe before administration. See product labeling for additional information. Use within 8 hours of reconstitution.

Storage/Stability

Fesilty: Store intact vials at 2°C to 30°C (36°F to 86°F) in original carton to protect from light. Do not freeze. Discard partially used vials. Store reconstituted solution at room temperature and use within 4 hours.

Fibryga: Store intact vials at 2°C to 25°C (36°F to 77°F) in original carton for up to 48 months from the date of manufacture; do not freeze. Protect from light. Discard partially used vials. Reconstituted solution should be used within 4 hours; do not refrigerate or freeze.

RiaSTAP: Store intact vials at 2°C to 8°C (36°F to 46°F) in original carton; do not freeze. Protect from light. Discard partially used vials. Reconstituted solution is stable for 8 hours when stored at 20°C to 25°C (68°F to 77°F).

Use: Labeled Indications

Acquired fibrinogen deficiency (Fibryga): Fibrinogen supplementation in bleeding pediatric and adult patients with acquired fibrinogen deficiency.

Congenital fibrinogen deficiency (Fesilty, Fibryga, Riastap): Treatment of acute bleeding episodes in pediatric and adult patients with congenital fibrinogen deficiency, including afibrinogenemia and hypofibrinogenemia.

Metabolism/Transport Effects

None known.

Drug Interactions

(For additional information: Launch drug interactions program)

There are no known significant interactions.

Reproductive Considerations

Patients with congenital fibrinogen deficiency may have an increased risk of bleeding, thrombosis, and pregnancy loss; replacement therapy may be initiated prior to conception in patients who could become pregnant (RCOG [Pavord 2017]).

Pregnancy Considerations

No human data exists to identify drug-associated risks of major birth defects, pregnancy loss, or other adverse maternal or fetal outcomes with use of fibrinogen concentrate during pregnancy.

Pregnant patients with congenital fibrinogen deficiency may have an increased risk of bleeding, thrombosis, and pregnancy loss; therefore, close surveillance is recommended. Maternal fibrinogen concentrations increase during pregnancy but do not protect against potential complications. Prophylaxis with fibrinogen concentrate throughout pregnancy may be needed; higher doses may be required as pregnancy progresses. Plasma derived fibrinogen concentrate may be used for treatment or prevention of bleeding in patients with severe deficiency (RCOG [Pavord 2017]).

Breastfeeding Considerations

It is not known if fibrinogen concentrate is present in human milk.

According to the manufacturer, the decision to continue or discontinue breastfeeding during therapy should take into account the risk of infant exposure, the benefits of breastfeeding to the infant, and benefits of treatment to the mother.

Monitoring Parameters

Fibrinogen level; viscoelastic properties of the fibrin-based clot; signs/symptoms of hypersensitivity and thrombosis.

Reference Range

A target fibrinogen level of 100 mg/dL (SI: 2.9 micromole/L) should be maintained until hemostasis occurs and wound healing is complete.

Normal fibrinogen levels: 200 to 450 mg/dL (SI: 5.9 to 13.2 micromole/L)

Mechanism of Action

Fibrinogen (coagulation factor I), a protein found in normal plasma, is required to clot blood. It leads to the formation of fibrin monomers that undergo polymerization and are stabilized by activated factor XIII. Factor XIIIa forms cross links between fibrin polymers which support the platelet plug and achieve hemostasis. Fibrinogen concentrate made from pooled human plasma replaces this protein which is missing or reduced in patients with a congenital or acquired fibrinogen deficiency.

Pharmacokinetics (Adult Data Unless Noted)

Note: In pediatric patients (12 to 17 years of age) receiving Fibryga, pharmacokinetic values were reported to be similar to adult data. In children 1 to <12 years of age, lower in vivo recovery, faster clearance, and shorter half-life were observed.

Distribution: V_d:

Fesilty: Patients 18 to 75 years of age: V_{dss}: 59.2 mL/kg.

Fibryga:

Patients <6 years of age: 68.6 ± 4.4 mL/kg (range: 63.9 to 72.7 mL/kg).

Patients 6 to <12 years of age: 67.2 ± 8.2 mL/kg (range: 52.8 to 76.8 mL/kg).

Patients ≥12 years of age (including adults): 70.2 ± 29.9 mL/kg (range: 36.9 to 149.1 mL/kg).

RiaSTAP: Patients 8 to 61 years of age: 52.7 ± 7.48 mL/kg (range: 36.22 to 67.67 mL/kg).

Half-life elimination: Similar to biological fibrinogen.

Biological fibrinogen: 100 hours (Kamath 2003).

Fesilty: Patients 18 to 75 years of age: 53.8 hours.

Fibryga:

Patients <6 years of age: 56.9 ± 10.8 hours (range: 45.6 to 67 hours).

Patients 6 to <12 years of age: 66.1 ± 12.1 hours (range: 57.7 to 91.6 hours).

Patients ≥12 years of age (including adults): 75.9 ± 23.8 hours (range: 40 to 157 hours).

RiaSTAP:

Patients <16 years of age: 69.9 ± 8.54 hours.

Patients ≥16 years of age: 82.3 ± 20.04 hours.

Excretion:

Clearance:

Fesilty: Patients 18 to 75 years of age: 0.801 mL/hour/kg.

Fibryga:

Patients <6 years of age: 0.9 ± 0.1 mL/hour/kg (range: 0.8 to 1 mL/hour/kg).

Patients 6 to <12 years of age: 0.7 ± 0.1 mL/hour/kg (range: 0.5 to 0.9 mL/hour/kg).

Patients ≥12 years of age (including adults): 0.7 ± 0.2 mL/hour/kg (range: 0.4 to 1.2 mL/hour/kg).

RiaSTAP: Patients 8 to 61 years of age: 0.59 ± 0.13 mL/hour/kg (range: 0.45 to 0.86 mL/hour/kg); subgroup analysis showed faster clearance in subjects <16 years of age.

Patient Counseling Points

(For additional information see "Fibrinogen, concentrate from human plasma: Patient drug information")

What is this drug used for?

• It is used to stop bleeding in patients with a certain type of blood clotting disorder (fibrinogen deficiency).

All drugs may cause side effects. However, many people have no side effects or only have minor side effects. Call your doctor or get medical help if any of these side effects or any other side effects bother you or do not go away:

• Feeling tired or weak

• Throwing up

• Back pain

WARNING/CAUTION: Even though it may be rare, some people may have very bad and sometimes deadly side effects when taking a drug. Tell your doctor or get medical help right away if you have any of the following signs or symptoms that may be related to a very bad side effect:

• Kidney problems like unable to pass urine, change in how much urine is passed, blood in the urine, or a big weight gain

• Liver problems like dark urine, tiredness, decreased appetite, upset stomach or stomach pain, light-colored stools, throwing up, or yellow skin or eyes

• Infections (parvovirus B19, hepatitis A) like fever or chills, headache, feeling very sleepy, runny nose, rash, joint pain, tiredness, poor appetite, upset stomach or throwing up, diarrhea, stomach pain, or yellow skin or eyes

• Weakness on 1 side of the body, trouble speaking or thinking, change in balance, drooping on one side of the face, or blurred eyesight

• Severe dizziness or passing out

• Fast or abnormal heartbeat

• Mood changes

• Confusion

• Fast breathing

• Blood clot like chest pain or pressure; coughing up blood; shortness of breath; swelling, warmth, numbness, change of color, or pain in a leg or arm; or trouble speaking or swallowing

• Signs of an allergic reaction, like rash; hives; itching; red, swollen, blistered, or peeling skin with or without fever; wheezing; tightness in the chest or throat; trouble breathing, swallowing, or talking; unusual hoarseness; or swelling of the mouth, face, lips, tongue, or throat.

Note: This is not a comprehensive list of all side effects. Talk to your doctor if you have questions.

Brand Names: International

International Brand Names by Country

For country code abbreviations (show table)

- (AE) United Arab Emirates: Clottafact | Fibclot | Haemocomplettan p;
- (AR) Argentina: Fibrinogeno | Fibryga | Haemocomplettan p;
- (AT) Austria: Fibrinogen human | Fibryga | Haemocomplettan p;
- (AU) Australia: Haemocomplettan p | Riastap;
- (BE) Belgium: Fibclot | Fibrinogene | Fibryga | Riastap;

- (BG) Bulgaria: Fibryga | Haemocomplettan;
- (BR) Brazil: Fibryga | Haemocomplettan p;
- (CH) Switzerland: Clottafact | Fibryga | Haemocomplettan p;
- (CL) Chile: Fibryga;
- (CN) China: Fibrinogen human | Fibroraas | Human Fibrinogen;
- (CO) Colombia: Clottafact | Fibryga | Haemocomplettan p;
- (CZ) Czech Republic: Fibrinogen immuno | Fibrinogen lidsky | Fibryga;
- (DE) Germany: Fibclot | Fibrinogen | Fibryga | Haemocomplettan hs | Haemocomplettan p;
- (EC) Ecuador: Fibryga;
- (EE) Estonia: Fibryga | Haemocomplettan p;
- (ES) Spain: Fibclot | Fibryga | Riastap;
- (FI) Finland: Fibclot | Fibryga | Riastap;
- (FR) France: Clottafact | Clottagen | Fibryga | Riastap;
- (GB) United Kingdom: Fibclot | Fibryga | Riastap;
- (GR) Greece: Haemocomplettan p | Riastap;
- (GT) Guatemala: Fibryga;
- (HK) Hong Kong: Fibrinogen human | Haemocomplettan p;
- (HU) Hungary: Fibrinogen | Fibrinogen human | Fibryga | Haemocomplettan p;
- (IE) Ireland: Fibryga | Riastap;
- (IT) Italy: Fibriclotte | Fibryga | Haemocomplettan p | Riastap;
- (JP) Japan: Fibrinogen mitsubishi;
- (KR) Korea, Republic of: Gcc fibrinogen;
- (LB) Lebanon: Haemocomplettan;
- (LT) Lithuania: Fibrema | Fibrinogen | Fibrinogen lidsky | Fibryga;
- (LV) Latvia: Fibrinogen | Fibrinogen lidsky | Fibryga;
- (MX) Mexico: Clottafact | Riastap;
- (MY) Malaysia: Fibryga;
- (NL) Netherlands: Fibclot | Fibryga | Haemocomplettan p;
- (NO) Norway: Fibclot | Fibryga | Haemocomplettan p | Riastap;
- (PL) Poland: Fibrinogen | Fibryga | Haemocomplettan hs | Haemocomplettan p | Riastap;
- (PR) Puerto Rico: Fibryga | Riastap;
- (PT) Portugal: Fibryga | Haemocomplettan;
- (PY) Paraguay: Fibryga;
- (QA) Qatar: Haemocomplettan P;
- (RO) Romania: Fibryga | Haemocomplettan p;
- (SA) Saudi Arabia: Haemocomplettan p;
- (SE) Sweden: Fibclot | Fibrinogen | Fibryga | Riastap;
- (SG) Singapore: Fibryga | Haemocomplettan p;
- (SI) Slovenia: Haemocomplettan p | Riastap;
- (SK) Slovakia: Fibryga | Riastap;
- (TN) Tunisia: Fibrinogen immuno | Haemocomplettan;
- (TR) Turkey: Haemocomplettan p;
- (TW) Taiwan: Fibrinogen | Haemocomplettan p;
- (UA) Ukraine: Fibryga;
- (UY) Uruguay: Fibryga

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Topic 9082 Version 173.0